

# **Prior Authorization Request**

## **Health Plan**

Humana Medicare Advantage -- Gold Choice PPO.

## **Member Information**

Member: Calvin O. Brightwater DOB: 1954-01-25 (age 72) Member ID: MBR-82265

Sex: M

## **Requesting Provider**

Practice: Summit Ridge Heart Institute

## **Requested Service**

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-11-04

Level of care requested: INPATIENT

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## EP consultation

SUMMIT RIDGE HEART INSTITUTE - Electrophysiology

Consult date 09/22/2026

Calvin O. Brightwater DOB 01/25/1954

Reason: ICD evaluation - EF decline on recent echo

HPI: 72M w/ ICM (anterior MI 05/2021 tx'd w/ LAD PCI). Was doing well in spring. Over the summer more DOE: now gets winded after about 2 blocks or one flight, okay with ADLs. No rest symptoms. No syncope. Wife reports no snoring change. No ACS or new coronary intervention since 2021.

Exam: BP 126/72, HR 64. Lungs clear. Trace ankle edema.

Data reviewed: TTE 09/02/2026 (report in chart). ECG SR, QRS 112.

Impression: ICM w/ progressive LV dysfunction. Current functional class: NYHA II.

Plan: primary prevention ICD if SDM supports. Pt will need inpatient stay, see plan.

Dr. Leona Farquharson

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## Shared decision making visit

Summit Ridge HF Program | SDM VISIT | 10/01/2026

Calvin Brightwater + wife Dolores

Clinician: Abdi Warsame, PA-C

Structured shared decision making for ICD per program protocol.

Decision aid used: published ICD decision aid (evidence-based), video + worksheet.

Pt top values: avoid sudden death, keep driving. Discussed 6-month driving restriction after shocks.

Decision: ICD. Pt and wife signed worksheet.

A. Warsame PA-C

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## Heart failure clinic note (prior)

Summit Ridge HF Clinic - Follow-up

Date: 06/10/2026

Pt: Brightwater, Calvin

S: Doing well. Walking 30 min daily w/o sx. No orthopnea.

O: BP 130/74, HR 62.

Data: TTE 03/15/2026 - LVEF 40%, anterior HK.

A: ICM, HFmrEF, NYHA I. Stable.

P: Continue current GDMT. Repeat echo in 3 mo. No ICD indication at this EF.

M. Odom, NP

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Medication list

MEDICATIONS - Brightwater, Calvin O. - as of 09/22/2026

carvedilol 25 mg BID ..... started 06/2021

valsartan 160 mg BID ..... started 06/2021

eplerenone 25 mg daily ..... started 09/2021

dapagliflozin 10 mg ..... started 03/2023

aspirin 81 mg daily ..... 2021

atorvastatin 80 mg ..... 2021

tamsulosin 0.4 mg qHS ..... 2019

warfarin per INR clinic ..... 2024 (LV apical thrombus 2024, resolved; anticoag continued per cards)

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Laboratory results

Summit Ridge Lab - 09/24/2026

Brightwater, Calvin

Na 136 | K 4.8 | Cr 1.6 H | eGFR 44 L

Hgb 13.2 | Plt 180

INR 2.6 (on warfarin)

NT-proBNP 1,980 H

A1c 6.1

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## Echocardiogram report

SUMMIT RIDGE HEART INSTITUTE - ECHOCARDIOGRAPHY

Date of exam: 09/02/2026

Patient: BRIGHTWATER, CALVIN O.

Reason: surveillance, ICM

LVEF (biplane): 30%

LV moderately dilated (LVIDd 6.2). Akinesis of the apex and distal anterior wall, hypokinesis of the anteroseptum.

No thrombus visualized.

Mod MR. RVSP 38.

Conclusion: Interval decline in LV function, LVEF now 30% (03/15/2026: 40%).

Read: P. Galloway, MD

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## Pre-admission plan

EP SCHEDULING AND ADMISSION PLAN

Date 10/02/2026 Brightwater, Calvin O. MBR-82265

Procedure: ICD, CPT 33249 Admit 11/04/2026 inpatient

LOS expected: 2 midnights.

Reasons: chronic warfarin, uninterrupted strategy (INR target 2-2.5), pocket hematoma surveillance; CKD 3b (eGFR 44); BPH w/ prior post-anesthesia urinary retention requiring catheter.

Limiting conditions checked: none (cognition intact, no malignancy, sinus rhythm/no AF, stable).

L. Farquharson MD

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